Fever / Infection Control

UP-10 | Universal Patient Care

Quick flow

01 Standard + Transmission-Based Precautions [EMT / AEMT / Paramedic]

Select contact, droplet, and/or airborne precautions by suspected route • mask respiratory-symptom patient when tolerated

02 Assess + Measure Temperature [EMT / AEMT / Paramedic]

Airway • breathing • perfusion • mental status • rash • source • temperature method • last antipyretic

03 Infant ≤60 Days + Temperature ≥100.4°F / 38°C? [EMT / AEMT / Paramedic]

Prompt transport and early notification • do not delay for antipyretics

04 Environmental Hyperthermia / Heat Stroke? [EMT / AEMT / Paramedic]

Transition to heat-emergency protocol • active cooling • antipyretics are not effective

05 Uncomfortable Fever ≥100.4°F / 38°C? [EMT / AEMT / Paramedic]

EMT/AEMT/Paramedic: one oral agent if airway protected and no contraindication

06 Suspected Infection + Organ Dysfunction? [EMT / AEMT / Paramedic]

Hypotension • altered mental status • respiratory distress/hypoxemia • poor perfusion → sepsis/shock pathway

07 Reassess + Transport [EMT / AEMT / Paramedic]

Repeat vital signs, temperature when useful, mental status, perfusion, respiratory status, and medication response

Overview

- Fever is a clinical sign, not a diagnosis. Identify the probable source, high-risk age or condition, evidence of organ dysfunction, environmental hyperthermia, and the need for transmission-based precautions.
- UP-10 permits oral acetaminophen or ibuprofen for an uncomfortable febrile patient who can safely take oral medication. Fever alone does not require vascular access, crystalloid, cardiac monitoring, or sepsis activation.

Provider-Level Actions

Higher provider levels include the actions listed for the preceding levels.

EMT

- Apply standard precautions and add contact, droplet, and/or airborne precautions according to the suspected route of transmission and agency infection-control policy. Place a mask on a patient with respiratory symptoms when tolerated and when it does not interfere with oxygen delivery or ventilation.
- Assess airway, respiratory effort, SpO_2, perfusion, mental status, skin, HEENT, neck, lungs, abdomen, back, neurologic status, hydration, rash, pain, and complete vital signs. Identify possible infectious source, environmental exposure, immune compromise, pregnancy, recent procedures, indwelling devices, sick contacts, and medications.
- Measure and document temperature and the measurement method. Do not perform rectal temperatures in the field. Accept a reliable documented temperature obtained at home or by another clinician even when antipyretics lowered the temperature before EMS arrival. Document the medication, dose, and administration time when known.

- Any infant 60 days old or younger with a documented temperature of at least 100.4°F/38°C requires prompt transport and early receiving-facility notification, even when well appearing. Do not delay transport to administer or assess the effect of an antipyretic.
- For an uncomfortable patient with temperature at least 100.4°F/38°C who is alert, protects the airway, can swallow safely, and has no medication contraindication, administer one oral antipyretic: acetaminophen or ibuprofen. Do not routinely administer both agents.
- Acetaminophen: adult 650-1,000 mg PO; pediatric older than 3 months 15 mg/kg PO, maximum 650 mg. Ibuprofen: adult 400 mg PO; pediatric older than 6 months 10 mg/kg PO, maximum 400 mg. Do not repeat an agent when the previous dose or administration time is uncertain.
- Do not administer aspirin to a patient younger than 16 years. Do not administer an antipyretic for environmental hyperthermia or heat stroke; transition immediately to the applicable heat-emergency protocol and begin active cooling.
- For seizure activity, protect the airway and transition to the seizure protocol. Do not delay transport of a toxic-appearing, unstable, immunocompromised, or high-risk patient for temperature reduction.

AEMT

- Do not establish IV/IO access or administer crystalloid for fever alone. Establish vascular access for hypotension, shock, critical illness, required parenteral treatment, or another specific clinical indication.
- For hypotension or shock, administer crystalloid according to UP-04: adult 500 mL and pediatric 10 mL/kg, followed by reassessment. Use smaller aliquots and frequent reassessment for CHF, renal failure, liver failure, or known volume overload.
- Obtain blood glucose for altered mental status, seizure, a critically ill infant or child, suspected sepsis with organ dysfunction, diabetes-related concern, or another specific indication. Routine glucose measurement is not required for uncomplicated fever.
- Administer the same oral acetaminophen or ibuprofen doses authorized above when indicated. UP-10 does not authorize IV acetaminophen, ketorolac, or antibiotic administration solely for fever.

Paramedic

- Fever alone is not sepsis. When suspected infection is accompanied by hypotension, altered mental status, respiratory distress or hypoxemia, poor skin perfusion, or another sign of organ dysfunction, transition immediately to the applicable sepsis and shock protocols.
- Cardiac monitoring is not required for uncomplicated fever. Apply and interpret cardiac monitoring for suspected sepsis with organ dysfunction, hypotension, significant respiratory distress, dysrhythmia symptoms, or other clinical instability.
- Manage airway or ventilatory failure, shock, seizure, dysrhythmia, or another identified complication under the applicable protocol. Notify the receiving facility early for sepsis, a high-risk infant, immunocompromise, meningismus, petechial or purpuric rash, or other time-sensitive findings.

Indications

- Measured or reliably documented temperature of at least 100.4°F/38°C, suspected infection, recent fever reduced by an antipyretic, or illness requiring infection-control precautions.

Contraindications

- Do not give oral medication to a patient who cannot protect the airway, cannot swallow safely, is actively vomiting, or has another aspiration risk.
- Do not administer acetaminophen with known hypersensitivity, severe liver disease or liver failure, suspected acetaminophen overdose, or a recent maximum dose.
- Do not administer ibuprofen with known NSAID hypersensitivity, age 6 months or younger, renal disease or renal transplant, active gastrointestinal bleeding, significant dehydration, hypotension, clinically significant anticoagulant-associated bleeding risk, or known or suspected pregnancy.
- Do not administer aspirin to a patient younger than 16 years.

Assessment | Focused fever and infection assessment

- Onset, duration, highest measured temperature and method, last antipyretic medication/dose/time, recent antibiotics, immune compromise, diabetes, cancer or chemotherapy, transplant, pregnancy, recent hospitalization or procedure, indwelling device, sick contacts, travel or environmental exposure, and suspected source.
- Assess for cough, dyspnea, chest pain, headache, neck stiffness, photophobia, sore throat, dysuria, flank pain, abdominal pain, vomiting, diarrhea, myalgias, rash, focal skin infection, altered mental status, rigors, seizure, poor intake, decreased urine output, and signs of dehydration.
- Identify organ dysfunction: hypotension, altered mental status, respiratory distress, hypoxemia, poor perfusion, mottling, delayed capillary refill, or rapidly worsening clinical appearance.

Treatment steps

- Use standard precautions and add transmission-based precautions according to the suspected route. Mask a patient with respiratory symptoms when tolerated.
- Measure and document temperature and method. Do not perform rectal temperatures. Treat a reliable home or clinician-documented fever as valid even when the current temperature is lower after an antipyretic.
- Transport an infant 60 days old or younger with documented temperature at least 100.4°F/38°C promptly with early notification; do not delay for antipyretic administration or effect.
- For uncomfortable fever, administer one oral agent when safe: acetaminophen adult 650-1,000 mg or pediatric older than 3 months 15 mg/kg to a maximum 650 mg; or ibuprofen adult 400 mg or pediatric older than 6 months 10 mg/kg to a maximum 400 mg.
- Do not establish vascular access, administer crystalloid, obtain routine glucose, or apply routine cardiac monitoring for uncomplicated fever. Use these interventions only for the approved clinical indications above.
- If environmental hyperthermia or heat stroke is suspected, begin active cooling and transition to the heat-emergency protocol. If suspected infection is accompanied by organ dysfunction, transition to the sepsis/shock pathway.
- Repeat complete vital signs and reassess mental status, airway, respiratory status, perfusion, temperature when useful, and medication response without delaying transport.

Medications

Acetaminophen - oral fever treatment

EMT/AEMT/Paramedic: adult 650-1,000 mg PO; pediatric older than 3 months 15 mg/kg PO, maximum 650 mg.

- Use for uncomfortable fever of at least 100.4°F/38°C when the patient protects the airway and can swallow safely.
- Do not administer with severe liver disease or liver failure, suspected acetaminophen overdose, known hypersensitivity, a recent maximum dose, or an uncertain previous dose/time.
- Do not delay transport of a high-risk infant, toxic-appearing patient, or unstable patient to administer or assess the effect of acetaminophen.

Ibuprofen - oral fever treatment

EMT/AEMT/Paramedic: adult 400 mg PO; pediatric older than 6 months 10 mg/kg PO, maximum 400 mg.

- Use for uncomfortable fever of at least 100.4°F/38°C when the patient protects the airway and can swallow safely.
- Do not administer with NSAID hypersensitivity, age 6 months or younger, renal disease/transplant, active GI bleeding, significant dehydration, hypotension, clinically significant anticoagulant-associated bleeding risk, pregnancy, or an uncertain previous dose/time.
- Do not administer for environmental hyperthermia or heat stroke.

Crystalloid - hypotension or shock only

AEMT/Paramedic: adult 500 mL; pediatric 10 mL/kg. Reassess after each bolus and use smaller aliquots for fluid-sensitive patients.

- Do not administer crystalloid for fever alone.
- Continue treatment according to UP-04 and the applicable sepsis or shock protocol.

Warnings / precautions

- Do not equate fever alone with sepsis. Sepsis requires suspected infection plus organ dysfunction; transition promptly when organ dysfunction is present.
- Infants 60 days old or younger with a documented temperature of at least 100.4°F/38°C require prompt evaluation even when currently afebrile or well appearing.
- Petechial or purpuric rash, meningismus, altered mental status, hypotension, hypoxemia, poor perfusion, toxic appearance, or immune compromise requires prompt transport and early notification.
- Antipyretics do not treat heat stroke and must never delay active cooling. Do not give NSAIDs in environmental heat emergencies.
- Do not delay transport of an unstable or high-risk patient to obtain a temperature, vascular access, cardiac monitoring, or antipyretic response.

Clinical pearls

- Treat the patient rather than the temperature. The purpose of an antipyretic is relief of discomfort; normalization of temperature is not required before transport.
- A normal temperature after acetaminophen or ibuprofen does not exclude serious infection. Document the highest reliable temperature and the medication, dose, and time given before EMS arrival.
- Antipyretics do not reliably prevent recurrence of febrile seizures. Treat active seizure activity under the seizure protocol.
- The absence of fever does not exclude infection or sepsis, especially in older, immunocompromised, or critically ill patients.
- Document suspected source, immune status, PPE used, temperature and method, medication contraindication screening, administered dose, response, vital-sign trends, and receiving-facility notification.

Special populations | Infants 60 days old or younger

- A reliably documented temperature of at least 100.4°F/38°C within the current illness is high risk even when the infant appears well or is afebrile after medication.
- Provide prompt transport and early notification. Do not delay transport for antipyretic administration or response.

Special populations | Immunocompromised patients

- Maintain a low threshold for prompt transport and early notification in patients with chemotherapy, cancer, transplant, HIV with immune suppression, chronic immunosuppressive medication, or another significant immune deficit.
- Serious infection may be present without marked fever or classic inflammatory signs.

Special populations | Pediatrics - younger than 16 years

- Use actual or length-based weight for medication dosing. Acetaminophen requires age older than 3 months; ibuprofen requires age older than 6 months.
- Do not administer aspirin. Assess hydration, urine output, interaction, work of breathing, capillary refill, rash, and caregiver-reported behavior.

Source and references

Tennessee EMS BLS/ALS State Protocol Guidelines 2024-2025 - Fever / Infection Control procedure and acetaminophen/ibuprofen references | American Academy of Pediatrics, Clinical Practice Guideline: Evaluation and Management of Well-Appearing Febrile Infants 8 to 60 Days Old, updated 2021 | Centers for Disease Control and Prevention - Standard and Transmission-Based Precautions, current guidance | Claiborne Covenant EMS Formulary - current medical-director source workbook | Claiborne County EMS UP-10 source PDF - Fever / Infection Control, revised September 1, 2025 | Claiborne County EMS UP-04 Shock / Hypotension and applicable sepsis, seizure, and environmental heat protocols

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